

Chapter 5 – Attachment and Psychopathology (Part 1)

I. An Overview of Attachment and Psychopathology

- Problems with attachment are viewed as “distal risk factors” for psychopathology.
- Attachment problems, either anxious or avoidant, make conditions ripe for “proximal risk factors” that link attachment with psychopathological disorders.
- Proximal risk factors in anxious attachment are (1) maladaptive emotion-regulation processes, with a tendency to up-regulate negative affectivity; (2) a greater vigilance to threat-related cues and heightened empathic accuracy; and (3) a lower level of perceived others’ responsiveness—that is, seeing others as less responsive and supportive and as less understanding of one’s needs.
- Proximal risk factors in avoidant attachment are (1) maladaptive emotion-regulation processes, with a tendency to down-regulate affectivity and employ distancing strategies; (2) compulsive self-reliance; and (3) lower levels of social support and perceived others’ responsiveness
- It takes specific “moderators,” interacting with the above proximal risk factors, to instigate psychopathology: (1) living in an environment with a chronically “mild to moderate” level of threat, (2) experience of significant loss, (3) traumatic experiences, and (4) the kind of “modeling, observational learning, and reinforcement that increase the likelihood of conduct problems among people high on anxiety or avoidance.”
- An illustration using the moderator of loss is a person who suffers a significant relationship breakup. In an anxiously attached person, the felt sense of loss and affective response might be more acute and intense and produce depressive symptoms.
- An avoidantly attached person might also experience depressive symptoms related to loss, but in reaction to a different kind or degree of loss that interacts with a proximal risk factor such as excessive self-reliance.
- A problematic attachment status and its proximal risk factors, *in the absence of a moderator*, do not necessarily result in psychopathology.
- Beyond disorders such as separation anxiety and pathological grief in which attachment injuries are the main causes and themes, attachment insecurities per se are unlikely to be sufficient causes of mental disorders. Other factors (e.g., genetically determined temperament; intelligence; life history, including abuse) are likely to converge with or amplify the effects of attachment experiences on the way to psychopathology.
- Serious mental disorders result from multiple converging processes, and attachment insecurities act as catalysts of other pathogenic processes by reducing psychological and social resources, and weakening a person’s resilience. Although attachment insecurities can contribute to psychological disorders, mental afflictions can also exacerbate attachment insecurity and lead to more severe attachment-system dysfunctions.

II. Self-Report vs. Interview Assessment of Attachment

- This chapter explores what is known about the empirical associations between insecure attachment classification and the risk for certain types of psychopathology. When reviewing this connection, it is essential to consider that attachment classifications are established by interview or self-report measures.
- Self-report measures, such as the Adult Attachment Scale (AAS) and Experiences in Close Relationships (ECR) scale, have a social and personality psychology heritage and rely on the subject's conscious and deliberate endorsement of attachment-related attitudes and behaviors. The AAI comes from the developmental psychology tradition and relies upon inferences about a subject's attachment style gleaned from the interview.
- Interview-based assessment is the preferred method over self-report measures, and the AAI allows for more detailed subclassifications, which is important for complex and severe cases. The AAI is able to precisely determine the individual's state of mind with respect to attachment and provide information about the IWMs. Studies looking at attachment and psychopathology that use self-report are still useful, particularly when they identify hyperactivating or deactivating strategies.

II. Attachment, Emotional Distress, and Psychiatric Distress

- 61 studies were reviewed that examined the relationship between attachment and psychiatric symptoms and negative affectivity. These studies measured different qualitative aspects of attachment – sense of security with peers and parents – using the Inventory of Parent and Peer Attachment (IPPA) and Parental Attachment Questionnaire (PAQ), as well as the AAS. All studies that used the IPPA or the PAQ found secure attachment to parents to be associated with lower levels of negative affectivity and less severe psychiatric symptomology.
- An overwhelming number of the studies found significant correlation between anxious attachment and psychiatric symptom/negative affectivity, whereas less than half found such correlations with the avoidant style.

Attachment and Anxiety Disorders

- Anxious and preoccupied states as measured by both the AAI and by self-report measures correlate with more significant symptoms of anxiety. Less clear association is found between avoidant style and anxiety symptoms (only 50% of studies found a significant association).
- In a study of attachment status using self-report (ECR) and OCD, results showed a relationship between insecure attachment and elevated levels of OCD-related thinking style and symptoms, with attachment anxiety having a somewhat stronger relationship. “Hyperactivation” of the attachment system may connect increased vigilance to intrusive thoughts, in particular those that are ego-dystonic, thereby increasing the need to act. Avoidant individuals with their exaggerated emphasis on autonomy, may be more vulnerable to acquiring OCD-related dysfunctional beliefs about being able to control their thoughts and achieving an unrealistically perfectionistic standard of behavior.
- In a noteworthy study of Israeli college students divided into spider-phobics and non-phobics, those with spider-phobia showed less aversion to spider pictures following secure attachment visualization and more aversion following insecure visualization. Findings indicated attachment insecurities are involved in the

generation of phobic symptomatology and restoration of attachment security can have healing effects on phobic people.

- Bowlby suggested all anxiety disorders are related to the proximity and availability of the primary attachment figure. Researchers comment that “the infant–caregiver attachment relationship is the womb from which the incipient person emerges. The first organization is dyadic, and it is from that organization, and not from the inborn characteristics of the infant, that personality emerges” and “a specific link was found between anxious/resistant attachment in infancy and anxiety disorders assessed 16 years later.”
- In the Minnesota longitudinal study converging 3 decades, the authors reported that insecure attachment in infants was associated with anxiety symptoms in adolescence, most likely due to the insecure internal working models of relationships.
- A study of 18 mothers with anxiety disorders (panic disorder, GAD, OCD) found 78% were determined to be unresolved with respect to trauma or loss (Ud). 85% of the children were classified as insecure and 65% were disorganized. Helping anxious adults resolve the losses and traumatic experiences of the past may indirectly benefit their children by improving the parent–child relationship. Recognizing Ud in clinically anxious populations can both enhance the treatment planning and efficacy, and indirectly benefit the insecurely attached children of the anxious patients.
- If IWMs play an important role in the etiology of anxiety disorders, it follows that psychotherapeutic treatment focused on reshaping these representations would lessen the suffering of patients with these disorders.

III. Attachment and Affective Disorders

Affective Disorders and Suicidality

- A review of over 100 studies of attachment quality in adulthood and depressive symptoms showed a strong correlation between levels of secure attachment and lower levels of depression.
- Those with either a dismissing or preoccupied classification on the AAI and self-report showed more depressive symptoms. Those with anxious attachment – both self-report and AAI – showed consistently elevated levels of symptoms of anxiety and depression compared to secure. In contrast, 50% of avoidant subjects across studies showed elevated levels of anxiety and depression.
- The AAI was given to 132 inpatient psychiatric patients with and without a history of suicidal ideation. Of the suicidal group, 73% had unresolved status with respect to trauma (Ud). 44% of the non-suicidal group had unresolved status with respect to trauma. The authors found that both preoccupied attachment and unresolved status were significantly associated with suicidal ideation, while dismissing attachment meant low suicidal ideation.
- In another study using the AAI, unresolved adults were considerably more likely to report emotional distress and suicidal ideation, dismissing adults were more likely to report criminal charges, and preoccupied adults were more likely to report suicidal ideation than either secure or dismissing adults. Subjects with unresolved status for trauma or loss reported greater suicidal ideation, emotional distress, and substance abuse than those with resolved status.

Attachment and Bipolar Disorder

- In a review of studies of attachment status in patients with psychosis – schizophrenia, bipolar disorder (type I), and psychotic depression – they found higher levels of insecure attachment, dismissing attachment in particular. Patients with bipolar psychosis were found to have less secure attachment than schizophrenics.
- Dismissing attachment was associated with an externalizing/outward focus in bipolar patients. Psychotic patients with hyperactivating (anxious) strategies reported greater symptom severity, but psychotic patients with dismissing strategies actually had more symptoms.
- While most studies using the AAI identified the dismissing classification as overrepresented in bipolar (mostly type I) subjects, an overrepresentation of disorganization was found in bipolar type II subjects.

IV. Attachment and Somatic Symptom Disorders, Factitious Disorders, and Malingering

Somatic Symptom Disorders

- Somatoform disorders are characterized as the existence of multiple and variable physical symptoms without demonstrable pathophysiological processes.
- In a study of a large sample of female primary care patients who were assessed for somatization, attachment style was significantly related to symptom reporting and health care utilization. Women with an anxious-preoccupied or fearful attachment style reported significantly more physical symptoms than secure women. Anxious-preoccupied women had a significantly greater number of primary care visits and higher health care costs than other women, and fearfully attached women had the lowest number of health care visits.
- In a study of 37 subjects diagnosed with various forms of somatoform disorder, significantly more patients than controls were classified as insecure on the AAI. Authors stated individuals with dismissing attachment minimize affects and convert affects into somatic symptoms.
- In a study of 28 children with conversion disorder using the Dynamic Maturation Model (DMM) assessment system, attachment style was clearly related to the type of conversion symptom: Negative CD symptoms, such as loss of sensory or motor functioning, were related to an inhibitory attachment style, whereas positive CD symptoms, such as symptom exaggeration, exaggerated seizure-like activity, limb weakness, and astasia-abasia, were all related to an excitatory attachment style.
- There is continuing controversy about whether fibromyalgia is a somatoform disorder, but a study using the AAI to assess attachment in 34 patients with fibromyalgia found unresolved status with respect to trauma was unusually high (50%) in these patients.

Factitious Disorders

- Factitious disorders are differentiated from somatoform disorders based on the voluntary production of physical symptoms.

- A factitious disorder is best defined as an “artificial production or simulation of a disease or history.” Comparable to malingering, factitious behavior involves deliberate deception and, often, intentional self-harm. Unlike malingering, factitious disorders are not obviously linked to environmental goals such as monetary gain or other incentives.
- In a study of 22 factitious patients, 61% self-reported attachment problems, 24% had rape histories, 19% had incest histories, and 28% had suffered traumatic object loss.
- In an important empirical study of 18 factitious patients, they found that nine had a coexisting borderline personality diagnosis and another six had a coexisting narcissistic personality diagnosis. 83% had significant evidence for “disorders of self-regulation” and it was concluded, “The overwhelming majority of patients suffering from factitious disorders therefore demonstrate a narcissistic pathology.”
- Some experts have viewed factitious behavior as an adaptive attempt to prevent total fragmentation of an unstable sense of self by asserting and reinforcing false identities.

Malingering

- In the orphanage study discussed later in the chapter, the Malingering Probability Scale (MPS) and the Structured Interview of Reported Symptoms (SIRS) inventories were given. Those with insecure (mainly disorganized) attachment were significantly related to an increased probability of exaggerating or fabricating psychiatric and/or physical symptoms.
- “We believe that, as a result of an insecure attachment status, factitious individuals, and also certain individuals who malingering, exaggerate or fabricate psychiatric symptoms based on a need to seek attention from caregivers. Many of the factitious patients we have studied have insecure attachment and histories of chronic or prolonged physical illnesses in childhood” . . . and attending by medical caretakers partially satisfied unmet attachment needs, and over time ultimately learned to compulsively produce physical or psychiatric symptoms.

V. Trauma-Related Disorders

Posttraumatic Stress Disorder

- A study of 112 women who were victims of incest during childhood, using the Relationship Questionnaire (RQ), found: 14% secure, 13% preoccupied, 16% dismissing, and 58% fearful. With respect to posttraumatic symptoms, subjects reporting secure attachment had the least avoidance in the Impact of Event Scale, consistent with Main and Goldwyn’s observation that adults assessed as securely attached had easier access to both positive and negative memories of childhood.
- A study of two groups: 85 veterans who fought and were held captive and 72 veterans who had similar combat exposure but who had not been captives, as well as their wives, found that among the veterans held captive, 24.8% showed clinically significant PTSD symptoms while 14.1% of their wives showed significant STS symptoms. In the noncaptive control group, 3.8% of the men exhibited significant PTSD symptoms and none of their wives showed significant secondary symptoms. Regarding attachment, in both study groups, higher attachment anxiety in the veterans and their wives was correlated with the severity of PTSD and STS, and only in the couples with a former captive soldier was avoidant status of the veterans and their wives associated with PTSD and STS.

- A study investigated the relationship between adult attachment style and “psychological adaptation” in a sample of 45 subjects present at the World Trade Center terrorist attack, a large number of which had direct exposure to graphic scenes. Subjects with secure attachment “exhibited relatively modest initial levels of PTSD symptoms that decreased over time.”

- From their review of over 200 studies of adult attachment that included over 10,000 AAI administrations, it was found that adults with abuse experiences or PTSD were mostly unresolved, and preoccupied attachment predicted unresolved status. Across studies, maternal unresolved attachment classification, maternal frightened/frightening behavior, and infant attachment disorganization were correlated.

- “It is of vital clinical importance to provide treatment that not only moderates the individual’s traumatic experiences and symptomology but also provides interventions aimed at decreasing the caregivers’ frightened/frightening behaviors and increasing their sensitivity and attunement-focused behaviors when engaging with their children.”

Attachment and Dissociative Disorders

- In a study of how attachment style and exposure to trauma relates to dissociative symptoms, authors state the main finding was that self-reported attachment style (both avoidance and anxiety) correlated substantially with dissociation, and more strongly than the number of self-reported potentially traumatic events. Authors also suggested that secure attachment might indeed protect against dissociation.

Depersonalization/Derealization Disorder (DRD)

- In the orphanage study of 17 adults who were sexually abused during childhood, 12 identified as having clinically significant depersonalization and 7 as having clinically significant derealization. An AAI primary classification of insecure (mostly disorganized) attachment was significantly associated with depersonalization and derealization, whereas subjects with an AAI primary classification of secure attachment showed few of these symptoms. Apparently, secure attachment status protected children who were sexually abused from developing depersonalization or derealization in adulthood.

Dissociative Identity Disorder (DID)

- Is defined by the existence of two or more alter personality states, loss of executive control when an alter personality state manifests over the host personality, and amnesia for a least some of the experiences/behaviors of alter personality states. Research has shown that somatoform symptoms are highly comorbid with DID.

- Personality functioning in DID patients can be broadly classified into an apparently normal personality structure (ANP) concerned with everyday functioning at the expense of emotional processing, and a number of emotional personality (EP) states concerned with remembering and processing stressful emotional and traumatic experiences.

- In a study of 60 AAI transcripts of DID patients, 90% had the classification CC as their primary attachment classification and 90% had Ud with respect to trauma or loss. These data strongly suggest that pervasive disorganization (CC), and specifically disorganization around the exploration of trauma or abuse (Ud), essentially defines the mind of the DID patient. “In the context of attachment, we have come to understand each alter personality state as an expression of a specific unmet attachment need in a patient with severe disorganized attachment, and the switches from one alter personality state to another personality state as sequential manifestations of these unmet attachment needs.”